

TEAM Eligibility - Synthetic Test Patient Bundle

Five synthetic patients designed to exercise all six TEAM eligibility checks. All names, MBIs, addresses, and clinical details are fictitious.

Bundle Contents

#	Patient	Anchor Procedure	Expected Verdict	Failing Check
1	Margaret O'Sullivan	LEJR	ELIGIBLE	(none - all six PASS)
2	Robert Hayes	HIP_FEMUR	INELIGIBLE (Medicare Advantage)	not_ma
3	Patricia Lin	SPINAL_FUSION	INELIGIBLE (ESRD-basis entitlement)	not_esrd_basis
4	James Whitfield	CABG	INELIGIBLE (UMWA Health Plan)	not_umwa
5	Dorothy Chen	MAJOR_BOWEL	ELIGIBLE	(none - all six PASS)

How to use this bundle

Upload this single PDF in the doctor portal Add Patient flow under either **Single** (the system will scope to the first patient) or **Group** (preferred - the system fans out one patient per record section). Each patient page includes a CWF-style eligibility block with explicit Part A/B status and dates, MA enrollment, MSP indicator, ESRD basis, and UMWA flag - shaped so the extraction prompt can lift each field with a verbatim sourceExcerpt.

Margaret O'Sullivan

MRN MRN-100231 | DOB 1953-04-12 (age 72) | Sex Female

Document type: EHR Eligibility & Pre-Op Export | Generated for TEAM intake review

Demographics & Contacts

Address	412 Elmwood Ave, Cleveland, OH 44102
Phone	(216) 555-0142
Primary Care Provider	Dr. Helena Brooks, MD - Cleveland Internal Medicine
MBI (Medicare Beneficiary Identifier)	1EG4TE5MK73

Scheduled Procedure

Anchor procedure (TEAM category)	LEJR
Procedure	Right Total Knee Arthroplasty (Total Knee Replacement)
Scheduled surgery date	2026-06-15
Operating surgeon	Dr. Anil Krishnan, MD - Orthopedic Surgery
Facility	MetroHealth Main Campus, Cleveland OH

Insurance & Medicare Eligibility (Coverage as of Surgery Date)

Source: Medicare Common Working File (CWF) eligibility response and internal payer verification. Reviewed by patient access on file date.

Primary payer	Medicare (Original / Fee-for-Service)
Part A status	ACTIVE
Part A effective date	2018-04-01
Part A termination date	(no termination date on file)
Part B status	ACTIVE
Part B effective date	2018-04-01
Part B termination date	(no termination date on file)
Part C / Medicare Advantage enrollment	NO - patient is enrolled in Original Medicare. No Part C / Medicare Advantage plan on file.
MA contract ID (if applicable)	(none)
Medicare Secondary Payer (MSP) indicator	NO - Medicare is the primary payer. No MSP record on file (no workers comp, no automobile liability, no employer group health plan as primary, no black lung).
Secondary / supplemental payer	AARP Medicare Supplement Plan G (UnitedHealthcare) - secondary only
Eligibility basis (CWF entitlement reason)	AGE (entitled at 65)
ESRD indicator	NO - patient does not have End-Stage Renal Disease. No dialysis. No ESRD entitlement on file.
UMWA Health Plan indicator	NO - patient is not enrolled in the United Mine Workers of America Health Plan.

Internal eligibility note: Expected TEAM verdict on review = **ELIGIBLE**. (This line is included for QA testing only.)

Active Problem List & Medical History

- Osteoarthritis, right knee (M17.11) - severe, refractory to conservative care
- Hypertension, controlled (I10) - lisinopril 20 mg daily
- Hyperlipidemia (E78.5) - atorvastatin 40 mg nightly
- GERD (K21.9) - omeprazole 20 mg daily
- BMI 28.4 - overweight, stable
- Cataract extraction OD (2022) - uneventful

Current Medications

- Lisinopril 20 mg PO daily
- Atorvastatin 40 mg PO QHS
- Omeprazole 20 mg PO daily
- Acetaminophen 500 mg PO Q6H PRN pain

Allergies

Penicillin (rash). No latex allergy.

Social History

Retired schoolteacher. Lives with spouse. Non-smoker. Occasional wine.

Recent Labs & Studies

- HbA1c 5.4% (2026-04-02)
- BMP within normal limits (2026-04-02)
- Hgb 13.2 g/dL, Plt 248k (2026-04-02)
- ECG: NSR, no ST changes (2026-04-15)

Pre-Operative Instructions

The following instructions are specific to Right Total Knee Arthroplasty (Total Knee Replacement) on 2026-06-15.

1. STOP eating solid food at midnight the night before surgery (NPO after midnight).
2. Clear liquids permitted up to 2 hours before scheduled arrival.
3. HOLD lisinopril on the morning of surgery. Resume POD #1 unless otherwise instructed.
4. Continue atorvastatin and omeprazole as scheduled, including morning of surgery (sip of water).
5. Shower with chlorhexidine (CHG) 4% wash the night before AND morning of surgery. Do not apply lotions, deodorants, or perfumes.
6. Arrive at MetroHealth Main Campus pre-op holding at 0530 on 2026-06-15 (case time 0730).
7. Bring CPAP if used at home, current medication list, photo ID, insurance card.
8. Arrange a responsible adult driver and 24-hour postoperative companion.
9. Pre-op PT visit completed 2026-05-20: home set up for safe ambulation, raised toilet seat in place.
10. Anticoagulation plan: aspirin 81 mg PO daily x 35 days starting POD #1 (DVT prophylaxis).

End of patient record. All data is synthetic; for software testing only.

Robert Hayes

MRN MRN-100447 | DOB 1955-09-22 (age 70) | Sex Male
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Demographics & Contacts

Address	88 Lakeshore Dr, Tampa, FL 33606
Phone	(813) 555-0987
Primary Care Provider	Dr. Carlos Mendoza, MD - Tampa Bay Primary Care
MBI (Medicare Beneficiary Identifier)	2W4FG7HK24

Scheduled Procedure

Anchor procedure (TEAM category)	HIP_FEMUR
Procedure	Left Intertrochanteric Femur Fracture - ORIF (Cephalomedullary Nail)
Scheduled surgery date	2026-06-20
Operating surgeon	Dr. Sarah Whitman, MD - Orthopedic Trauma
Facility	Tampa General Hospital

Insurance & Medicare Eligibility (Coverage as of Surgery Date)

Source: Medicare Common Working File (CWF) eligibility response and internal payer verification. Reviewed by patient access on file date.

Primary payer	Humana Gold Plus HMO (Medicare Advantage / Part C)
Part A status	ACTIVE (administered through Medicare Advantage)
Part A effective date	2020-10-01
Part A termination date	(no termination date)
Part B status	ACTIVE (administered through Medicare Advantage)
Part B effective date	2020-10-01
Part B termination date	(no termination date)
Part C / Medicare Advantage enrollment	YES - Patient is enrolled in HUMANA GOLD PLUS HMO, a Medicare Advantage Prescription Drug (MAPD) plan. MA enrollment is active as of surgery date.
MA contract ID (if applicable)	H1036 (Humana Medical Plan, Inc. - Florida HMO)
Medicare Secondary Payer (MSP) indicator	N/A - patient receives Medicare benefits through Part C plan. Plan-level MSP rules apply.
Secondary / supplemental payer	(none)
Eligibility basis (CWF entitlement reason)	AGE (entitled at 65; enrolled in MA at 65)
ESRD indicator	NO - no End-Stage Renal Disease. No dialysis history.
UMWA Health Plan indicator	NO - not enrolled in UMWA Health Plan.

Internal eligibility note: Expected TEAM verdict on review = **INELIGIBLE (Medicare Advantage)**. (This line is included for QA testing only.)

Active Problem List & Medical History

- Mechanical fall at home 2026-06-18, LEFT hip pain, unable to bear weight
- Closed left intertrochanteric femur fracture (S72.142A) - confirmed on plain film + CT
- Type 2 Diabetes Mellitus (E11.9) - HbA1c 6.8%, on metformin
- Coronary artery disease s/p DES to LAD 2019 - on dual antiplatelet completed 2020
- Atrial fibrillation, paroxysmal - on apixaban (HELD on admission)
- Hypertension - amlodipine 10 mg daily

Current Medications

- Metformin 1000 mg PO BID
- Amlodipine 10 mg PO daily
- Apixaban 5 mg PO BID (HELD - last dose 2026-06-18 0800)
- Atorvastatin 80 mg PO QHS
- Acetaminophen 1 g PO Q6H scheduled
- Oxycodone 5 mg PO Q4H PRN severe pain

Allergies

NKDA

Social History

Retired electrician. Lives alone in single-story home. Former smoker (quit 2010).

Recent Labs & Studies

- HbA1c 6.8% (2026-05-30)
- Cr 1.0 mg/dL, GFR 78 (2026-06-18)
- Hgb 11.4 g/dL, Plt 195k (2026-06-18)
- INR 1.1 (off apixaban x 48h)
- ECG: paroxysmal AFib, rate-controlled

Pre-Operative Instructions

The following instructions are specific to Left Intertrochanteric Femur Fracture - ORIF (Cephalomedullary Nail) on 2026-06-20.

1. URGENT/EMERGENT case - target OR within 24-48h of fracture.
2. NPO since 2026-06-18 2200; clear liquids until 2 hours pre-op.
3. HOLD apixaban; last dose 2026-06-18 0800. Cleared by cardiology for surgery 2026-06-20.
4. Continue amlodipine and atorvastatin morning of surgery with sip of water.
5. Insulin sliding scale per inpatient protocol; HOLD metformin morning of surgery.
6. CHG wipes night before and morning of surgery.
7. Type and cross 2 units PRBC; have available in OR.
8. DVT prophylaxis: pneumatic compression stockings on admission; restart apixaban POD #1 per surgeon.
9. Geriatrics co-management consult - in place.

10. Coordinate weight-bearing status and rehab placement at discharge.

End of patient record. All data is synthetic; for software testing only.

Patricia Lin

MRN MRN-100612 | DOB 1962-07-08 (age 63) | Sex Female

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Demographics & Contacts

Address	27 Maple Ridge Ln, San Jose, CA 95126
Phone	(408) 555-0331
Primary Care Provider	Dr. Yusuf Erkan, MD - South Bay Nephrology & IM
MBI (Medicare Beneficiary Identifier)	3K8HJ9MN56

Scheduled Procedure

Anchor procedure (TEAM category)	SPINAL_FUSION
Procedure	Posterior Lumbar Interbody Fusion L4-L5 (PLIF)
Scheduled surgery date	2026-07-10
Operating surgeon	Dr. Marcus Lefevre, MD - Spine Surgery
Facility	Stanford Health Care - Valley Specialty Center

Insurance & Medicare Eligibility (Coverage as of Surgery Date)

Source: Medicare Common Working File (CWF) eligibility response and internal payer verification. Reviewed by patient access on file date.

Primary payer	Medicare (Original / Fee-for-Service)
Part A status	ACTIVE
Part A effective date	2020-08-01
Part A termination date	(no termination date)
Part B status	ACTIVE
Part B effective date	2020-08-01
Part B termination date	(no termination date)
Part C / Medicare Advantage enrollment	NO - Original Medicare. No Part C / Medicare Advantage on file.
MA contract ID (if applicable)	(none)
Medicare Secondary Payer (MSP) indicator	NO - Medicare is primary. ESRD 30-month coordination period ended 2023-02; Medicare has been primary since.
Secondary / supplemental payer	(none active)
Eligibility basis (CWF entitlement reason)	ESRD (End-Stage Renal Disease) - patient qualifies for Medicare BECAUSE OF ESRD, not age or disability. Eligibility basis on CWF: ESRD entitlement effective 2020-08-01. ESRD is the qualifying condition.
ESRD indicator	YES - Eligibility basis: ESRD. Patient on chronic hemodialysis 3x/week. ESRD entitlement is the pathway by which patient receives Medicare. (Note: this is distinct from ESRD as a comorbidity in an age-entitled beneficiary.)
UMWA Health Plan indicator	NO - not enrolled in UMWA Health Plan.

Internal eligibility note: Expected TEAM verdict on review = **INELIGIBLE (ESRD-basis entitlement)**. (This line is included for QA testing only.)

Active Problem List & Medical History

- End-Stage Renal Disease (N18.6) - on hemodialysis MWF since 2020-08
- Medicare entitlement based on ESRD diagnosis (qualifying event 2020-05); patient is under 65
- Diabetes Mellitus Type 2 - longstanding, complicated by ESRD
- Hypertensive nephrosclerosis
- Lumbar spondylolisthesis L4-L5 with severe canal stenosis - failed 9 months conservative care
- Anemia of CKD - on darbepoetin
- Secondary hyperparathyroidism - on cinacalcet

Current Medications

- Insulin glargine 24 units SubQ QHS
- Insulin aspart sliding scale with meals
- Amlodipine 10 mg PO daily
- Carvedilol 12.5 mg PO BID
- Sevelamer 800 mg PO TID with meals
- Cinacalcet 30 mg PO daily
- Darbepoetin alfa 60 mcg SubQ weekly (dialysis days)
- Atorvastatin 40 mg PO QHS

Allergies

Iodinated contrast (hives). NKDA otherwise.

Social History

Retired accountant. Lives with daughter. Non-smoker. No alcohol.

Recent Labs & Studies

- Pre-dialysis BUN 62, Cr 7.4 mg/dL (2026-07-01)
- K 5.0, Phos 5.6, Ca 8.9 (2026-07-01)
- Hgb 10.3 g/dL on darbepoetin (2026-07-01)
- iPTH 412 pg/mL (2026-06-15)
- HbA1c 7.4% (2026-06-10)
- Dialysis access: left forearm AVF, mature, in use

Pre-Operative Instructions

The following instructions are specific to Posterior Lumbar Interbody Fusion L4-L5 (PLIF) on 2026-07-10.

1. Schedule dialysis the day before surgery (2026-07-09). NO dialysis morning of surgery.
2. NPO after midnight. Clear liquids permitted up to 2 hours pre-op.
3. HOLD insulin glargine to half-dose (12 units) the night before; HOLD all aspart on morning of surgery.

- 4.** HOLD amlodipine and carvedilol morning of surgery; resume POD #1 per anesthesia.
- 5.** Continue sevelamer with any oral intake; HOLD on day of surgery.
- 6.** Protect left forearm AVF: NO blood pressures, IVs, or blood draws on left arm. Sign on bed.
- 7.** Renal anesthesia consult required. Avoid nephrotoxic agents (NSAIDs, IV contrast, aminoglycosides).
- 8.** Type and cross 2 units PRBC. Anemia management: target Hgb >10 pre-op.
- 9.** CHG wipes night before AND morning of surgery.
- 10.** Arrange post-op dialysis on POD #1 or #2 per nephrology.
- 11.** Discharge planning: coordinate outpatient dialysis at usual center.

End of patient record. All data is synthetic; for software testing only.

James Whitfield

MRN MRN-100819 | DOB 1948-11-30 (age 77) | Sex Male

Document type: EHR Eligibility & Pre-Op Export | Generated for TEAM intake review

Demographics & Contacts

Address	1402 Coal Ridge Rd, Beckley, WV 25801
Phone	(304) 555-0264
Primary Care Provider	Dr. Linda Hargrove, MD - Beckley Family Medicine
MBI (Medicare Beneficiary Identifier)	4PQ7RT8VW01

Scheduled Procedure

Anchor procedure (TEAM category)	CABG
Procedure	Coronary Artery Bypass Grafting x3 (LIMA-LAD, SVG-OM1, SVG-RCA)
Scheduled surgery date	2026-06-25
Operating surgeon	Dr. Henry Okonkwo, MD - Cardiothoracic Surgery
Facility	WVU Heart and Vascular Institute, Morgantown WV

Insurance & Medicare Eligibility (Coverage as of Surgery Date)

Source: Medicare Common Working File (CWF) eligibility response and internal payer verification. Reviewed by patient access on file date.

Primary payer	Medicare (Original / Fee-for-Service)
Part A status	ACTIVE
Part A effective date	2013-12-01
Part A termination date	(no termination date)
Part B status	ACTIVE
Part B effective date	2013-12-01
Part B termination date	(no termination date)
Part C / Medicare Advantage enrollment	NO - Original Medicare. No Medicare Advantage / Part C plan on file.
MA contract ID (if applicable)	(none)
Medicare Secondary Payer (MSP) indicator	NO - Medicare is the primary payer.
Secondary / supplemental payer	UMWA HEALTH AND RETIREMENT FUNDS - 1974 Pensioners Benefit Plan (UMWA Health Plan). Plan ID: UMWA-1974-RET. Effective 2014-01-01. Provides supplemental coverage for retired coal miners.
Eligibility basis (CWF entitlement reason)	AGE (entitled at 65)
ESRD indicator	NO - patient does not have ESRD. No dialysis.

UMWA Health Plan indicator

YES - patient is enrolled in the United Mine Workers of America Health Plan (UMWA 1974 Pensioners Benefit Plan) as a retired coal miner. Plan is active and confirmed via 270/271 inquiry on 2026-06-01.

Internal eligibility note: Expected TEAM verdict on review = **INELIGIBLE (UMWA Health Plan)**. (This line is included for QA testing only.)

Active Problem List & Medical History

- Severe three-vessel coronary artery disease - cath 2026-05-29: LAD 90% prox, OM1 85%, RCA 80%
- Stable angina (CCS class III) on maximal medical therapy
- Hypertension (I10) - controlled
- Hyperlipidemia (E78.5)
- Type 2 Diabetes (E11.9) - HbA1c 7.0%
- Coal Workers' Pneumoconiosis (J60) - mild, stable on serial imaging
- Former coal miner x 32 years (Beckley area underground mining); retired 2014
- Tobacco use - former, quit 2008 (50 pack-years prior)

Current Medications

- Aspirin 81 mg PO daily
- Atorvastatin 80 mg PO QHS
- Metoprolol succinate 100 mg PO daily
- Lisinopril 20 mg PO daily
- Metformin 1000 mg PO BID
- Nitroglycerin 0.4 mg SL PRN angina

Allergies

NKDA

Social History

Retired underground coal miner (32 years). Married. Lives at home. Former smoker.

Recent Labs & Studies

- Hgb 13.8 g/dL, Plt 220k (2026-06-15)
- Cr 1.1 mg/dL, GFR 72 (2026-06-15)
- HbA1c 7.0% (2026-06-10)
- LDL 71, HDL 38, TG 142 (2026-06-10)
- ECG: NSR, old inferior Q waves
- Echo: LVEF 50%, mild MR, no significant valvular disease

Pre-Operative Instructions

The following instructions are specific to Coronary Artery Bypass Grafting x3 (LIMA-LAD, SVG-OM1, SVG-RCA) on 2026-06-25.

1. NPO after midnight 2026-06-24. Clear liquids until 2h pre-op.
2. Continue aspirin 81 mg through day of surgery.

3. HOLD metformin morning of surgery; resume POD #2 if eating.
4. HOLD lisinopril morning of surgery; resume POD #1-2 per surgeon based on hemodynamics and renal function.
5. Continue metoprolol and atorvastatin morning of surgery (sip of water).
6. CHG wipes nightly x 3 nights pre-op AND morning of surgery (full body, avoid eyes/ears).
7. Pre-op clipping (NOT shaving) of chest, abdomen, and bilateral legs morning of surgery.
8. Pulmonary toilet: incentive spirometry teaching pre-op given coal workers' pneumoconiosis.
9. Pulmonary function testing complete 2026-06-08: mild restrictive pattern, FEV1 78% predicted.
10. Type and cross 2 units PRBC. Cell saver available intraop.
11. Carotid duplex: <50% bilateral - cleared.
12. Dental clearance: completed 2026-05-22, no active infection.
13. Discharge planning: cardiac rehab referral; UMWA plan covers Phase II program.

End of patient record. All data is synthetic; for software testing only.

Dorothy Chen

MRN MRN-101032 | DOB 1950-02-14 (age 76) | Sex Female

Document type: EHR Eligibility & Pre-Op Export | Generated for TEAM intake review

Demographics & Contacts

Address	318 Cedar Hollow Rd, Asheville, NC 28803
Phone	(828) 555-0119
Primary Care Provider	Dr. Priya Ramaswamy, MD - Blue Ridge Internal Medicine
MBI (Medicare Beneficiary Identifier)	5XB6CD7EF89

Scheduled Procedure

Anchor procedure (TEAM category)	MAJOR_BOWEL
Procedure	Laparoscopic Sigmoid Colectomy (for sigmoid adenocarcinoma, T2N0)
Scheduled surgery date	2026-07-05
Operating surgeon	Dr. Olivia Bertrand, MD - Colorectal Surgery
Facility	Mission Hospital, Asheville NC

Insurance & Medicare Eligibility (Coverage as of Surgery Date)

Source: Medicare Common Working File (CWF) eligibility response and internal payer verification. Reviewed by patient access on file date.

Primary payer	Medicare (Original / Fee-for-Service)
Part A status	ACTIVE
Part A effective date	2015-02-01
Part A termination date	(no termination date)
Part B status	ACTIVE
Part B effective date	2015-02-01
Part B termination date	(no termination date)
Part C / Medicare Advantage enrollment	NO - Original Medicare. No Part C / Medicare Advantage plan enrollment on file.
MA contract ID (if applicable)	(none)
Medicare Secondary Payer (MSP) indicator	NO - Medicare is the primary payer. No workers compensation, no automobile liability, no employer group health plan as primary, no black lung benefits.
Secondary / supplemental payer	Aetna Medicare Supplement Plan G (Medigap) - secondary supplemental only; Medicare remains primary.
Eligibility basis (CWF entitlement reason)	AGE (entitled at 65)
ESRD indicator	NO - patient does not have End-Stage Renal Disease. No dialysis history. (Renal function: Cr 0.9, GFR 70.)
UMWA Health Plan indicator	NO - patient is not enrolled in the United Mine Workers of America Health Plan.

Internal eligibility note: Expected TEAM verdict on review = **ELIGIBLE**. (This line is included for QA testing only.)

Active Problem List & Medical History

- Sigmoid adenocarcinoma (C18.7), T2N0 - found on screening colonoscopy 2026-05-12, biopsy confirmed 2026-05-15
- Hypertension (I10) - well-controlled
- Osteoporosis (M81.0) - on alendronate weekly
- Hypothyroidism (E03.9) - on levothyroxine, euthyroid
- Mild diverticulosis (incidental)
- Cholecystectomy 2008 (laparoscopic, uneventful)

Current Medications

- Levothyroxine 75 mcg PO daily
- Amlodipine 5 mg PO daily
- Hydrochlorothiazide 12.5 mg PO daily
- Alendronate 70 mg PO weekly
- Calcium carbonate 600 mg + Vitamin D3 800 IU PO daily

Allergies

Sulfa drugs (rash). NKDA otherwise.

Social History

Retired librarian. Widowed. Lives alone, has supportive daughter nearby. Non-smoker. Rare alcohol.

Recent Labs & Studies

- CEA 2.4 ng/mL (2026-06-20)
- CT chest/abd/pelvis: sigmoid mass, no metastases (2026-05-22)
- Staging MRI pelvis: T2N0 (2026-05-25)
- Hgb 12.6 g/dL, Plt 232k (2026-06-20)
- Albumin 4.0 g/dL (2026-06-20)
- TSH 1.8 (2026-04-10)

Pre-Operative Instructions

The following instructions are specific to Laparoscopic Sigmoid Colectomy (for sigmoid adenocarcinoma, T2N0) on 2026-07-05.

1. ERAS (Enhanced Recovery After Surgery) protocol - colorectal pathway.
2. Mechanical bowel prep with oral antibiotics 2026-07-04: Polyethylene glycol (4 L) afternoon/evening + neomycin 1 g + metronidazole 500 mg PO at 1300, 1400, and 2200.
3. Clear liquids only on 2026-07-04 starting 0800.
4. NPO from 2400 on 2026-07-04. Carbohydrate-loading drink (Ensure Pre-Surgery, 12 oz) at 0400 on 2026-07-05.
5. HOLD hydrochlorothiazide morning of surgery; resume POD #1 if hemodynamically stable.
6. Continue amlodipine, levothyroxine morning of surgery (sip of water).
7. HOLD alendronate; do not resume until tolerating full diet and able to remain upright 30 min.

- 8.** CHG wipes night before AND morning of surgery.
- 9.** Pre-op nutrition consult complete; albumin >3.5, low malnutrition risk.
- 10.** DVT prophylaxis: SCDs on admission + enoxaparin 40 mg SubQ daily starting 2h pre-op.
- 11.** Arrange responsible adult to drive home; expect 3-4 day inpatient stay.
- 12.** Post-op: early ambulation POD #0 evening, oral diet advance per ERAS, multimodal non-opioid analgesia.

End of patient record. All data is synthetic; for software testing only.